

Medical Necessity Criteria: Heart Failure Inpatient Admission (DRG 291/292)

Policy Number: POL-CARD-2024-008 | Category: Medical Necessity | Department: Medical Management

1. PURPOSE

This policy defines medical necessity criteria for inpatient admission of members presenting with congestive heart failure (CHF) or acute decompensated heart failure (ADHF), billed under DRG 291 (Heart Failure & Shock with MCC) or DRG 292 (without MCC).

2. MEDICAL NECESSITY CRITERIA FOR INPATIENT ADMISSION

Any ONE of the following clinical indicators supports inpatient admission:

- O2 saturation < 90% on room air or requiring supplemental O2 > 2L/min
- Heart rate > 120 bpm or < 50 bpm with hemodynamic compromise
- Systolic BP < 90 mmHg (cardiogenic shock criteria)
- BNP > 400 pg/mL or NT-proBNP > 1800 pg/mL on presentation
- New or worsening renal failure (Cr increase > 0.5 mg/dL from baseline)
- Requiring IV diuresis or vasoactive medications

3. EXPECTED LENGTH OF STAY

Uncomplicated ADHF (DRG 292): 3-5 days

ADHF with MCC (DRG 291): 5-8 days

MCC for DRG 291 commonly includes:

- Septicemia (A41.x) as secondary diagnosis
- Acute respiratory failure (J96.0x)
- Acute kidney failure (N17.x)
- CKD Stage 4-5 (N18.4, N18.5) with acute decompensation
- Diabetes with hyperglycemia (E11.65) requiring IV insulin

4. COMORBIDITY CODING - RISK ADJUSTMENT IMPLICATIONS

The following secondary diagnoses affect HCC risk scores and must be coded accurately when present and treated during the admission:

HCC 85 - Congestive Heart Failure (I50.x):

RAF weight: 0.331. Must document LVEF, ejection fraction type (systolic/diastolic).

HCC 136 - Chronic Kidney Disease Stage 5 (N18.5):

RAF weight: 0.289. Stage must be documented by treating physician.

HCC 18 - Diabetes with Chronic Complications (E11.65):

RAF weight: 0.302. Hyperglycemia treated with IV insulin qualifies.

Medical Necessity Criteria: Heart Failure Inpatient Admission (DRG 291/292) (continued)

5. PRIOR AUTHORIZATION

Emergency inpatient admissions for ADHF do not require prior authorization.

Planned admissions for elective catheterization or device implant during CHF workup require prior authorization under POL-CARD-2024-012.

Continued stay reviews are conducted at Day 3 for DRG 291 admissions.

Continued stay criteria must be documented in progress notes.

6. AUDIT TRIGGERS

The following patterns trigger automatic claims audit routing:

- LOS > 10 days without documented complications or transfer note

- DRG 291 billed without documented MCC in claim diagnosis field

- HCC-qualifying diagnoses coded on claim but not documented in discharge summary

- BNP / NT-proBNP not documented in chart for DRG 291 admission

- Total charges > \$90,000 for standard CHF admission

7. DISCHARGE PLANNING

Discharge to home with HH services (FL 17 status 06): document HH order in chart.

Discharge to SNF (FL 17 status 03): 3-day qualifying inpatient stay required for Medicare.

Discharge to LTAC (FL 17 status 62): requires prior authorization.

8. REFERENCES

ACC/AHA 2022 Guideline for Heart Failure Management

InterQual Criteria: Acute Care, Cardiovascular - Heart Failure

CMS ICD-10-CM Official Guidelines FY2024, Section I.C.9

CMS-HCC Risk Adjustment Model V28 (2024)